Document Control

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Pre-operative Anaemia Guideline (Elective Hysterectomy)				
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2. Purpose

2.1. The purpose of this document is to define the process for management of patients who are found to be anaemic prior to elective hysterectomy.

2.2. The guideline applies to pre-op assessment, anaesthetic, surgical, haematology, pharmacy and primary care staff.

2.3. Implementation of this guideline will ensure that:

- Patients undergoing elective hysterectomy are not put at avoidable perioperative risk due to untreated pre-operative anaemia

3. Definitions

Hb

3.1. Haemoglobin level (g/L)

CRP

3.2. C-reactive protein

Anaemia (female patient)

3.3. Female Hb < 120 g/L

Iron deficiency

3.4. Anaemia with ferritin < 30 ug/L

Functional iron deficiency

3.5. Anaemia with ferritin < 100 and elevated CRP

4. Responsibilities

4.1. Implementation of this guideline will be the responsibility of the Lead Clinician and Lead Nurse for Pre-operative assessment in liaison with the Gynaecology team and other key stakeholders detailed below.

Role of Pre-op Assessment Nurse

4.2. The Pre-op Assessment Nurse is responsible for:

- Ensuring that patients with pre-operative anaemia are identified at the earliest possible opportunity
- Ensuring that patients with pre-operative anaemia are offered treatment as detailed in this guideline

- Ensuring that patients' GP practices are notified of any newly identified anaemia and of any initial treatment commenced or required
- Ensuring that patients treated for pre-operative anaemia have their haemoglobin level re-assessed prior to surgery

Role of the Pre-op Assessment Anaesthetist

4.3. The Pre-op Assessment Anaesthetist is responsible for:

- Providing specialist advice and support in the management of pre-operative anaemia

Role of the Haematology Consultant

4.4. The Haematology Consultant is responsible for:

- Providing specialist advice and support in the management of pre-operative anaemia

5. Pre-operative anaemia management

5.1. See flowchart in Appendix 6.1

6. Monitoring Compliance with and the Effectiveness of the Guideline

Standards/ Key Performance Indicators

6.1. Key performance indicators comprise:

- Haemoglobin levels at pre-op assessment prior to elective hysterectomy
- Final haemoglobin levels prior to elective hysterectomy
- Transfusion rates in elective hysterectomy
- Haemoglobin levels at discharge following elective hysterectomy
- Length of hospital stay following elective hysterectomy

Process for Implementation and Monitoring Compliance and Effectiveness

6.2. The flowchart in Appendix 6.1 will be utilised by the Pre-op assessment nurses to initiate appropriate patient management and the standard letters in Appendix 6.2 will be used by them to ensure prompt and accurate communication with primary care.

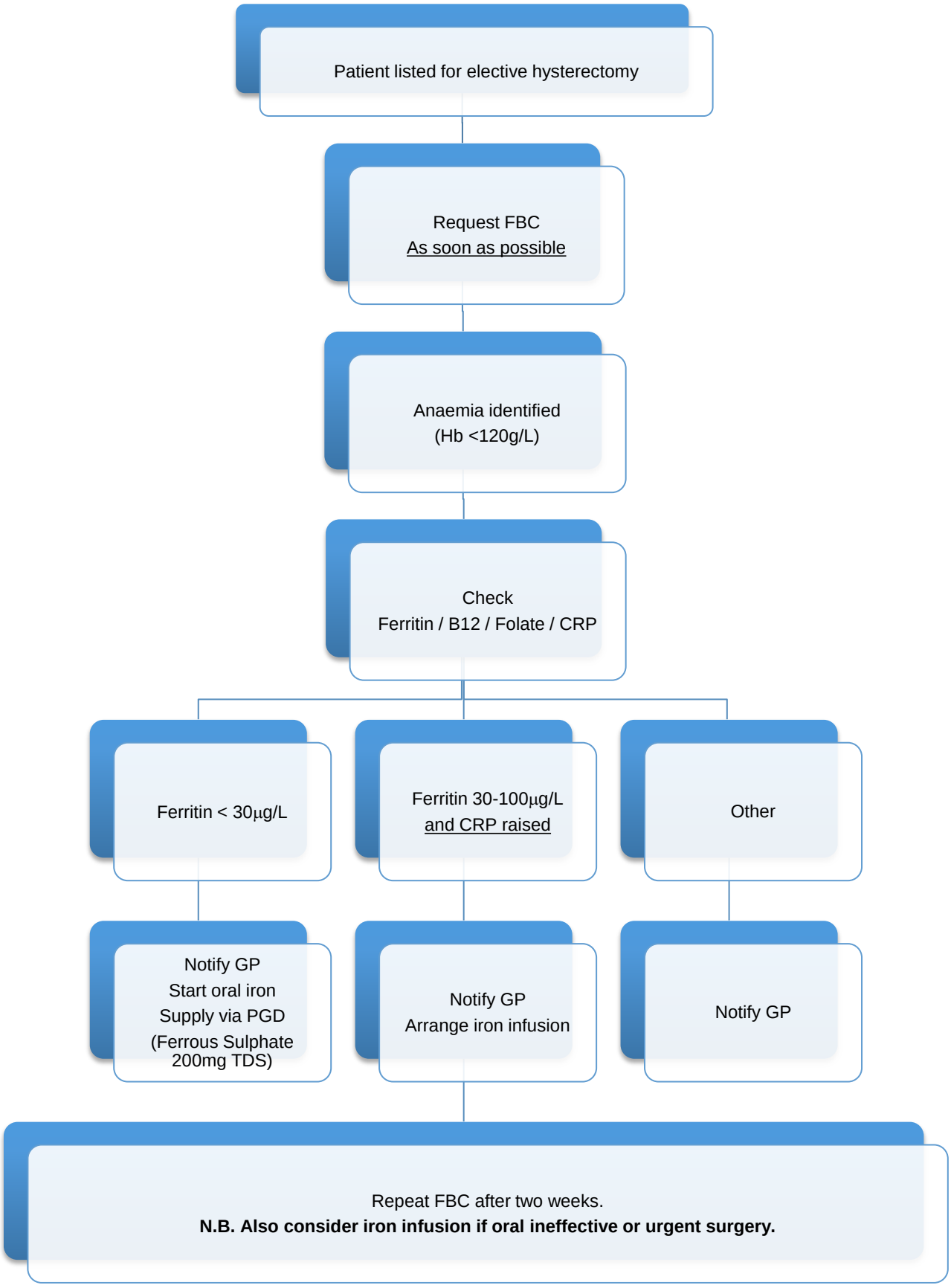
6.3. The process will be monitored internally within the Pre-op Assessment team and, more broadly, as a component part of the Patient Blood Management initiative via the Patient Blood Management Group

7. Appendices

Flowchart for the management of pre-operative anaemia

7.1. The following flowchart is intended to guide staff in the optimisation of patients who are found to be anaemic prior to elective hysterectomy (see next page):

Management of Anaemia Prior to Elective Hysterectomy



Standard letters for communication with primary care

7.2. The following standard letters are intended to facilitate safe and effective communication with primary care regarding the optimisation and management of patients who are found to be anaemic prior to elective hysterectomy (see next page):

7.2.1. GP letter for use with oral iron

NHS
Northern Devon Healthcare
NHS Trust

North Devon District Hospital
Raleigh Park
Barnstaple, Devon
EX31 4JB

Tel: 01271322577
Fax: 01271 311541
Minicom: 01271322746

www.northdevonhealth.nhs.uk

Dear Doctor,

Re. Patient Label Here

The above patient has been identified as being anaemic during their preparation for surgery.

- They have been started on oral iron due to evidence of iron deficiency (Ferritin <30µg/L)

Please consider any further investigations as necessary.

We have attached a copy of our pre-operative anaemia pathway by way of further explanation.

Kind regards,

Pre-Op Assessment Nurse
North Devon District Hospital

Chief Executive: Suzanne Tracey

Chair: James Brent

7.2.2. GP letter for use with iron infusion

NHS
Northern Devon Healthcare
NHS Trust

North Devon District Hospital
Raleigh Park
Barnstaple, Devon
EX31 4JB

Tel: 01271322577
Fax: 01271 311541
Minicom: 01271322746

www.northdevonhealth.nhs.uk

Dear Doctor,

Re.

Patient Label Here

The above patient has been identified as being anaemic during their preparation for surgery.

- An iron infusion has been arranged on the grounds of either:
  - Ferritin <30μg/L
  - Ferritin 30-100μg/L with elevated CRP

Please review their B12/folate results (if required commence treatment immediately) and consider any further investigations as necessary.

We have attached a copy of our pre-operative anaemia pathway by way of further explanation.

Kind regards,

Pre-Op Assessment Nurse
North Devon District Hospital

Chief Executive: Suzanne Tracey

Chair: James Brent

7.2.3. GP letter for use in other anaemias (i.e. no iron therapy)

NHS
Northern Devon Healthcare
NHS Trust

North Devon District Hospital
Raleigh Park
Barnstaple, Devon
EX31 4JB

Tel: 01271322577
Fax: 01271 311541
Minicom: 01271322746

www.northdevonhealth.nhs.uk

Dear Doctor,

Re.

Patient Label Here

The above patient has been identified as being anaemic during their preparation for surgery.

- Results suggest that they will not benefit from iron therapy pre-operatively on the basis of either:
  - Ferritin >100μg/L
  - Ferritin >30μg/L with normal CRP

Please review their B12/folate results (if required commence treatment immediately) and consider any further investigations as necessary.

We have attached a copy of our pre-operative anaemia pathway by way of further explanation.

Kind regards,

Pre-Op Assessment Nurse
North Devon District Hospital

Chief Executive: Suzanne Tracey

Chair: James Brent

Ferric derisomaltose infusion patient specific direction

7.3. The following form is intended for use in the prescription of an intravenous iron infusion in patients found to be anaemic prior to elective surgery:

Patient Label Here or:

Patient Name:

Date of Birth:

NHS Number:

Patients 50kg and over

Administer 1000mg of ferric derisomaltose in 100ml of Sodium Chloride 0.9% over 30 minutes.

Prescriber Signature: _____

Registration Number:_____

Date: ____/____/_____

Patients under 50kg - Maximum dose 20mg/Kg

Patient weight: ____Kg

Dose to be administered =

Patient weight x 20

_____Kg x 20 = _____mg

Administer _____mg of ferric derisomaltose in 100ml of Sodium Chloride 0.9% over 30 minutes.

Prescriber Signature: _____

Registration Number:_____

Date: ____/____/_____

Administered By: _____

Registration Number: _____

Date: _____

8. References

- British Committee for Standards in Haematology Guidelines on the Identification and Management of Pre-Operative Anaemia. *British Journal of Haematology*, 2015, **171**, 322-331
- International consensus statement on the peri-operative management of anaemia and iron deficiency. *Anaesthesia*, 2017, **72**, 233-247

9. Associated Documentation

- Iron Replacement Therapy Assessment of Clinical Competency, Northern Devon Healthcare Trust
- Patient Group Direction for the Supply of Ferrous Sulphate 200mg tablets by Registered Practitioners employed by Northern Devon Healthcare NHS Trust
- Monofer Infusion for iron deficiency in adults Standard Operating Procedure